**جوستاف روسي انترناشيونال - مصر (EPE) ***

# طلب علاج أول مرة / متردد

**إسم المريض رباعياً :** هند فاروق محمد سعداوي
**المحافظة :** القاهره **الحي / المدينة :** المعادي **العنوان :** --
**رقم البطاقة :** 27701150101981 **رقم التليفون :** 01285242907 **تاريخ تسجيل الإستمارة :** 10-10-2026
**العمر(تاريخ الميلاد) :** 15-01-1977 **المهنة :** ربه منزل

## الإقرار

**أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك**

**الإخصائي الاجتماعي** **المقر بما فيه**
**التوقيع:**(----------) **التوقيع:**(----------)

**التشخيص الإكلينيكي :** سرطان بالثدي (مبادرة)
**نتائج الأبحاث :** (مرفق)
**التشخيص النهائي :** سرطان بالثدي (مبادرة)
**خطة العلاج :** علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي الاورام المنتشر [14456] **قسم العلاج :** الاورام
**التعليق على التشخيص :** ----------

## تقرير اللجنة الثلاثية المتخصصة

**الاسم:** د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

**أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.**
**كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.**

**تعتمد صحة البيانات المرفقة**
**رئيس اللجنة و مدير المستشفي.**

**تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.**

## أعضاء اللجنة

**ا.د/** ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

## قرار اللجنة

**التشخيص النهائي :** ----------
**خطة العلاج :** ----------
**المستشفي :** ----------**قسم :** ----------
**جهة العلاج :** ----------**نفقات :** ----------
**ملاحظات :** ----------

| رئيس | مدير عام | ختم |
|---|---|---|
| المجلس الطبي المتخصص | مديرية الشئون الصحية | شعار الجمهورية |

Name: هند فاروق محمد سعداوي
National ID: 27701150101981

# تقرير طبي

السيد

تحية طيبة وبعد,,

| | |
|---|---|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم: | رقم طبي: |

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

| | |
|---|---|
| تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd | تاريخ وساعة الخروج : PM 01:17 05-07-2026 |

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Pin Medication And Anti Emetics

| | |
|---|---|
| التاريخ : 05-07-2026 | الوقت : -- --:-- |
| | الطبيب المعالج : د/شيماء لاشين |




على موجه
+
٢٠١٨٨

National ID · Front
[[DOCTYPE:ID_FRONT]]




National ID · Back
[[DOCTYPE:ID_BACK]]

| | |
|---|---|
| **Name:** هند فاروق محمد سعودي | |
| **MR. :** 2662 | |

Detailed Medical Report

**السيد**

**تحية طيبة وبعد,,**

| | | | |
|---|---|---|---|
| **إيماء للطلب المقدم من** | | **بشأن تقرير طبي للمريض** | |
| **بقسم:** | | **رقم طبي:** | |

**ننشرف بأن نوضح فيما يلي تقريرا عن حالته**

| | | | |
|---|---|---|---|
| **تاريخ وساعة الدخول :** | | **تاريخ وساعة الخروج :** | |

**وبعد الكشف والفحوصات والمتابعة وجد انه**

**MDT 9/2022: ZOLADEX>FEMARA> IBRANCE>ZOMETA CT 10/2025:SD BONE SCAN 10/2025:SD**

**last bone sca 12\1 rt breast mets to cervicolumbar vertebrae ,scrum, both sacroiliac rt ischium.**

**Clinical pharmacy note : for zometa today not received 7/12/2025 new labs OK**

**mbc pt is known diapetic 20/4/2026 mdt :pd for aromasine +affinitor approval paper done**

**affinitor+aromasin +zometa next clinic visit :8/7/2026**

**Patinet has increased pain over metastatic site (specifically SIJ) ,, For Palliative RTH over bil SIJ (previous pall RTH on dorsal vv 4 years ago) . Consider Zometa mnthly if pain persisted , Pain Clinic already on FU. for 3rd cycles Affinitor Aromasine today and Will do CT assessment after this cycle .**

**pt received palliative rth to both SIJ 20gy/5f/1w from 15/8 to 19/8 ct 30/7 >> multiple osseous lesion ( already present before no comparison ) for new bone scan compare to last one in 4/2026 for c4 afinitor + aromasin + zoladex zometa due 1/9/2026 at pharmacy after labs**

**تحريرا في ,**

| | | | |
|---|---|---|---|
| **التاريخ :** | 19-09-2026 | **الوقت :** | |

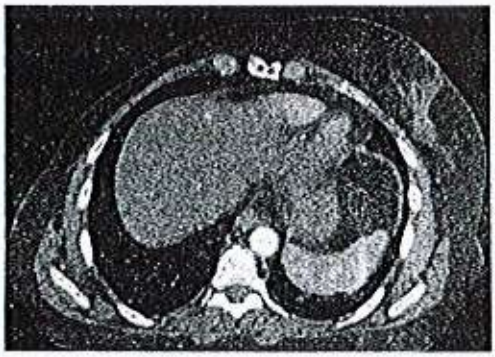
**الطبيب المعالج :** نهي سعيد علي محمد

**يعتمد ,**

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

| | |
|---|---|
| Name : Hend farouk mohamed saodi | Date : 04-02-2026 04:01 |
| Branch : Portable | Medical No. : GRI-12276 |
| Ref.Physician Prof : cap+c | Ins.Company : |

# *MULTI-DETECTOR C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS FOLLOWING ORAL AND INTRAVENOUS CONTRAST MEDIA ADMINISTRATION REVEALED:*



*Clinical data: Managed right breast cancer with MRM as compared with previous CT the current study reveled*

*-Stationary course with clear right mastectomy bed with no evidence of tumor recurrence*

*-Left lower calyceal two stones measures 7 mm and 480 HU with no back pressure changes*

*Otherwise;*

*-Normal lung parenchyma, showing no focal lesions.*

*- Normal vascular mediastinal structures.*

*- The trachea, main stem bronchi as well as their lobar and segmental divisions are normal.*

*- No evidence of mediastinal or hilar nodal enlargement .*

*- No pleural collections.*

*- The liver is of normal size, and tissue density. It shows no focal or enhancing lesions.*

*- The intra and extra-hepatic biliary radicles are normal.*

*- The pancreas and spleen are normal.*

*- Normal CT features of right kidney.*

*- Right and left supra renal glands are normal.*

*- No CT evidence of enlarged abdominal or pelvic lymph nodes.*

*- No ascites.*

*- Scans through the pelvis showed no masses.*

*Opinion*

- *Clear right mastectomy bed with no evidence of tumor recurrence*
- *Left lower calyceal stone*

*Much Obliged;*
*Dr.Khaled Moghazy,MD*
*Prof.of Radiology*

CT Scan
[[DOCTYPE:CT_SCAN]]

Name : Hind Farouk Mohammed Saud
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 12-01-2026 11:00
Medical No. : 293317
Ins.Company : تعاقد مستشفيات

**Whole Body Bone Scintigraphy (Technetium-99m MDP)**

**INDICATION:** 49-year-old female with history of metastatic breast cancer. Prior bone scan dated [16/04/2025] showed multifocal skeletal metastases with stationary course.

**TECHNIQUE:** Following intravenous administration of Technetium-99m MDP, planar anterior and posterior whole-body images were obtained approximately 3 hours post-injection.

**FINDINGS:**

Compared to the prior bone scan of [16/04/2025], there is interval progression of multifocal skeletal metastatic disease, manifested by both an increase in the number of lesions and an increase in the intensity of uptake in previously identified sites.

- **Newly seen osseous lesions:** small focal lesion at the right occipitoparietal aspect of the skull, as well as the sternum.
- **Old lesions showing increased number/tracer uptake intensity:** multiple cervicodorsolumbar vertebrae, sacrum, both sacroiliac articulations and right ischium.

**IMPRESSION:**

1. **Interval progression of multiple metastatic osseous lesions as detailed**, for clinical/radiological correlation.
2. Follow-up as warranted is kindly advised.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine



PET Scan
[[DOCTYPE:PET_SCAN]]

Name : Hind Farouk Mohammed Saud
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 12-01-2026 11:00
Medical No. : 293317
Ins.Company : تعاقد مستشفيات

NILE SCAN & LABS DIAGNOSTIC CENTER

**Whole Body Bone Scintigraphy (Technetium-99m MDP)**

**INDICATION:** 49-year-old female with history of metastatic breast cancer. Prior bone scan dated [16/04/2025] showed multifocal skeletal metastases with stationary course.

**TECHNIQUE:** Following intravenous administration of Technetium-99m MDP, planar anterior and posterior whole-body images were obtained approximately 3 hours post-injection.

**FINDINGS:**

Compared to the prior bone scan of [16/04/2025], there is interval progression of multifocal skeletal metastatic disease, manifested by both an increase in the number of lesions and an increase in the intensity of uptake in previously identified sites.

- **Newly seen osseous lesions:** small focal lesion at the right occipitoparietal aspect of the skull, as well as the sternum.
- **Old lesions showing increased number/tracer uptake intensity:** multiple cervicodorsolumbar vertebrae, sacrum, both sacroiliac articulations and right ischium.

**IMPRESSION:**

1. **Interval progression of multiple metastatic osseous lesions as detailed,** for clinical/radiological correlation.
2. Follow-up as warranted is kindly advised.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

PET Scan
[[DOCTYPE:PET_SCAN]]

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / هند الدروى محمد سعودى | Request No. | : 13009152 |
| Patient No. | : 27701150101981 | Request Date | : 30/08/2022 |
| Path No. | : SR-640/22 | Verification Date | : 19/09/2022 |
| Sex | : Female | Financial Category | : اورام دار السلام - فئة الدولة |
| Age | : 45 Y | Department | : Out Patient |
| Referreing Doc : | | Registered by | : marwa mohamed youss |

# Pathology Report

Clinical data: Known case of metastatic breast cancer to lung & bone presented by Right suprarenal mass.
Nature of Specimen : Slide revision (US guided biopsy from right suprarenal mass)
Gross: One block & one slide labeled as 676152.

Microscopic:
Metastatic adenocarcinoma
- IPT ( Positive control showed appropriate reactivity ) :
- GATA3: Positive nuclear staining
- Synaptophysin: Negative
- ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
- PR: Strong nuclear reaction in 60% of tumor cells (Allred score 7/8)
- HER2: Moderate complete membrane reaction in >10% of tumor cells (score 2+)
- Average Her2 copy No.: 2.2
- Her2/ CEP17 ratio: 1.6
- SISH: Not Amplified.

Diagnosis:
Right supra-renal mass, Slide revision, Metastatic carcinoma of mammry origin.
- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- HER2: Equivocal (score 2+).
- SISH: Not Amplified.

Prof.Dr.Iman Gouda
Prof. Lobna Omar
Dr. Marwa Mahmoud

Verified by :



Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

| | | | |
|---|---|---|---|
| Patient Name | : الاستاذ / هلا فاروق محمد سعودى | Request No. | : 13009152 |
| Patient No. | : 27701150101981 | Request Date | : 30/08/2022 |
| Path No. | : SR-640/22 | Verification Date | : 07/09/2022 |
| Sex | : Female | Financial Category | : اورام دار السلام - ثقة الدولة |
| Age | : 45 Y | Department | : Out Patient |
| Referreing Doc : | | Registered by | : .marwa mohamed youss |

## Pathology Report

Clinical data: Known case of metastatic breast cancer to lung & bone presented by Right suprarenal mass.

Nature of Specimen : Slide revision (US guided biopsy from right suprarenal mass)

Gross: One block & one slide labeled as 676152.

Microscopic:
Metastatic adenocarcinoma

Diagnosis:
Metastatic adenocarcinoma for IPT; GATA3, Synaptophysin, ER ,PR, HER2 )

Prof.Dr.Iman Gouda
Prof. Lobna Omar
Dr. Marwa Mahmoud

Verified by :
Dr.Marwa mahmoud



Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

| | |
|---|---|
| Patient Name: Hend Farouk Mohamed Saodi | Date : 08/10/2025 |
| Exam : CT CAP(CT Chest Abd. Pelvis) | Age : 48 Year(s) |
| Medical ID: 3138/2022 | Pat. ID: 12276 |

**NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS**

**REVEALED:**

- **Compared to the previous study dated 06/03/2025.**

1. **Chest:**
   - Right MRM with still no operative bed sizable mass lesions or collection.
   - **No gross time interval changes** regarding:
     - Few prominent left cardio-phrenic lymph nodes.
     - Few bilateral scattered tiny pulmonary nodules.
   - Otherwise, no pathological hilar or mediastinal lymph nodes.
   - No evidence of pleural effusion, pleural thickening or masses seen.
   - Average cardiac size. No pericardial effusion detected.
2. **Abdomen and pelvis:**
   - The liver is average in size with homogenous low parenchymal attenuation (fatty liver). No definite focal lesions
   - No intra-hepatic or extra-hepatic biliary radicles dilatation.
   - Stable right adrenal lesion measures about 2.6 x 3.8 cm.
   - Normal size & parenchymal texture of the spleen with no focal lesions seen.
   - No pathological abdominal or pelvic lymph nodes.
   - Normal size, parenchymal thickness & density of both kidneys with no right renal stones or back-pressure changes. Still noted left renal 1.5 cm stone (current density about 1415 HU) with mild left hydrocalycosis.
   - Normal appearance of the pancreas and left supra-renal gland.
   - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
   - Normal appearance of the other pelvic organs.
   - No abdominal or pelvic masses or collections.
   - IUD seen in situ.
   - Rather stationary wide spread axial and appendicular skeleton heterogenous sclerotic lesions & foci.

**OPINION:**

- **Follow up state running rather stationary course since last study dated 06/03/2025.**

Prof. Ali Remah



CT Scan
[[DOCTYPE:CT_SCAN]]

# MDT FORM 20 / 9 / 2022 G

| | | | |
|---|---|---|---|
| Name | [illegible] | MRN | 2022 / 3138 |
| | | ID | 27901150161981 |
| Age | 45 yrs | Phone | ٠٩٩٧٣٨٤٤٤ |
| Menopausal status | Premenop | marital status | married |
| NRS SCORE | (2) | family history | -ve |
| BMI | obese | REASON OF MDT | |
| Co morbidity | ·DM<br>RT MRM + cs | | |

## Clinical Data

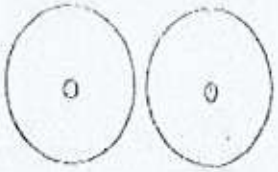
Bx: IDC II

| ER: | PR: | HER 2: | Ki67: |
|---|---|---|---|
| + | + | -ve | 25 % |

Sonomammography:
- US post operative RT MRM.
Free Postop bed

Mets Workup: cTs . Fu: Lt axillary LN
- Pulmo Nodules
- Rt adenexal mass
osseous dzp

R L



TNM

Recommendations: Adj EC 4 → HT? adenexal bx pending
-For AI then Progression
Plan for Sec Line Treat HT

MDT OUTCOME & PLAN

Paladex / Femara / Kisqali / Ibrance

CPRC(a)

| SIGNATURE | | | | | |
|---|---|---|---|---|---|
| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY | Nutrition | psychiatric |
| | | | | | |

20/9/2022 Angelsh